



PROCEDURE

Rights of Carers and Other Support Persons

Scope (Staff):	All NMHS Mental Health (MH) Staff: Clinical
Scope (Area):	All NMHS MH Areas

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1. Aim

NMHS Mental Health (NMHS MH) is committed to providing information, inclusion and recognition of the role that carers, family members and other support people play in supporting and providing care for consumers with a mental illness. Carers, families and support persons should be key partners in the delivery of ongoing care to consumers who have a disability, chronic illness, mental illness, or those who are frail. This procedure should be read in conjunction with the [NMHS Carers Policy](#).

2. Background

NMHS MH aligns with and is committed to the principles and values of the [Australian Charter of Healthcare Rights](#), the [WA Public Consumer Charter](#), the [Carers Recognition Act 2010](#) (including the Carers Charter), [Chief Psychiatrist's for Clinical Care Standard](#): Consumer and Carer Involvement in Individual Care, and the [Charter of Mental Health Care Principles](#) which, ensures the rights of personal support persons are respected by the health service. Personal support persons include parent/guardians, carers, close family members and nominated persons. The commitment includes ensuring personal support persons are made aware of their rights under the [Mental Health Act 2014](#).

3. Risk

The risks associated with non-compliance of this policy puts the psychological and mental health needs of carers and support persons in jeopardy and provides sub-optimal experiences and care for the consumers.

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

Care: All services and interventions provided to a person with a mental illness by health and other sectors, community organisations, family and carers.

Carer/s: Defined by [Carers Recognition Act 2004](#), a carer refers to an individual who provides ongoing care, support and assistance to a person with disability, a chronic illness (which includes mental illness) or who is frail, without receiving a salary or wage for the care provided.

Close Family Member: Defined by s.281 of the [Mental Health Act 2014](#) (refer to appendix 1).

Consumer/s: Are people who identify as having a living or lived experience of mental illness, irrespective of whether they have a formal diagnosis, have accessed services and/or received treatment. This includes people who describe themselves as a “peer”, “survivor” and “expert by experience”.

Enduring guardian and guardian of a child: A person appointed under an Enduring power of Guardianship (EPG) to make important personal, lifestyle and treatment decisions on behalf of the adult person. (Refer to the [Guardianship and Administration Act 1990](#) Part 9A)

Guardian: An adult appointed by the State Administrative Tribunal to make wide-ranging decision, including treatment decisions, on behalf of an adult person who does not have the capacity to make such decisions for him/herself (Refer to the [Guardianship and Administration Act 1990](#) Part 5).

Next of kin: Refers to a person who is aged 18 years or older and is the person’s spouse, de facto partner or closest living relative. *Note: if you have difficulty identifying who is the preferred person to contact as the next of kin, you may refer to the decision making hierarchy outlined in the [WA Health Consent to Treatment Policy](#).*

Nominated Person: Means a person nominated under s.263(1) of the Mental Health Act 2014. A nominated person may be the primary carer or any other person who is formally nominated by a person experiencing mental illness. A nominated person is entitled to be informed and involved whilst the consumer is being provided with treatment or care.

Paid carer or support workers: A person who is paid a salary or hourly rate to look after people with their care needs. This is **NOT** covered in this procedure.

Parent or Guardian: Means the person who has responsibility (as defined in the [Family Court Act 1997](#)) for the child.

Recovery: Is personal and means different things to different people. Personal recovery is defined within the National Framework for Recovery-Oriented Mental Health Services as *“being able to create and live a meaningful and contributing life in a community of choice with or without the presence of mental health issues”*.

Personal Support Person: means a close family member, carer, nominated person, parent or guardian of a child and guardian or enduring guardian of an adult who provides or has provided in the past, ongoing personal care, support, advocacy and/or assistance to a person with mental illness.

Young Carer: Defined by Carers WA, a “*young carer*” is a person between age 8 and 25 who helps support a member of their household who is experiencing a mental health challenge, disability, long term health condition (including a chronic condition or terminal illness), a substance dependency or who is frail aged.

For further clarification on the definitions, refer to the **Appendices 1 & 2**.

5. Principles

NMHS MH recognises, respects, values and supports the importance of carers and other support persons to the wellbeing, treatment and recovery of consumers. Where consumers are unable to understand their rights, their carers or other support persons must be made aware of and assisted to understand the consumer’s rights. Information should be provided at regular intervals throughout the consumer’s care and will comply with [MH Release of Patient Information Policy](#).

In accordance with Health Service (Information) Regulations 2017, where a patient requests that verbal or written information about their care be withheld from a carer, family member, next of kin, guardian, personal support person or any other third party then that information may not be divulged to the specified person.

It is a requirement of these guidelines that health clinicians share information with the carer and/or other support person/s that is necessary for their role.

Carers and other support persons have the right to ask for and be given general verbal and written information about:

- The mental health illness and diagnosis.
- What behaviours to expect and how to handle them.
- Medication - alternatives, benefits and possible side effects.
- Available local services for the consumer - both inpatient and community.
- Available local services for the carer – government, private and community.
- Local and national support groups.

Carers and other support persons can be given:

- Ongoing opportunities to ask questions, give feedback, and discuss concerns.
- The opportunity to see health clinicians on their own.
- The right to confidentiality in communications with and to a clinician.
- Positive feedback as a valued member of the care team.
- Confidence to express their opinion and concerns.
- Practical and emotional support.

Carers and other support persons can be given help in understanding:

- The rights and responsibilities of consumers, carers, and mental health clinicians.
- Any restrictions on sharing information requested by the consumer.
- The aims of, and what is involved in, the consumer's treatment plan.
- A written care and crisis plan and a recovery programme.
- The roles of each health clinician involved in care of the Consumer

Carers and other support persons may communicate to mental health clinicians:

- Any history of the consumer relevant to the mental illness and diagnosis.
- Personal characteristics of the consumer that may influence the choice of medication or treatment, e.g. behaviours, likes and dislikes.
- Observed ongoing benefits or side effects of medication or treatment on the consumer.
- Problems they have in caring for the consumer and when they need help in practical problem solving.

Carers, family members, nominated persons, and/or personal support persons must be contacted as soon as possible and be informed with respect to the consumer's care and treatment. Communication with carers and other support persons in all aspects of the consumer's care is mandatory as per the National Standards for Mental Health Services (2010) and the WA [Mental Health Act 2014](#).

6. Procedure

All NMHS MH staff have the following responsibilities:

- Ensure information on carers and other support persons rights and responsibilities is readily available and provided on request. (**Appendix 1**)
- Offer the use of interpreters, opportunity to ask questions, use patient advocates and involve carers, family members or support persons in the discussion of rights and responsibilities. Provide information to carers and other support persons in appropriate formats for those with cultural and linguistically diverse needs. Evidence of these actions should be included in the health record.

- Ensure information exchanges between the service, patient's carers and other support persons are age appropriate and use a variety of methods and media to assist consumers and carers and nominated persons to understand their rights.
- Ensure consumers and carers and other support persons are informed of and supported in easily accessing complaints and redress procedures.

In circumstances where consumers are unable to give consent (**See Appendix 2** on consent), carers and other support persons help exercise the following rights:

- To relay information concerning family relationships or other relevant information relating to the health of the consumer to the service.
- To seek further opinions regarding the care of the consumer.
- To access assistance with difficulties generated by the process of caring for and representing the consumer.
- To respect the consumer's individual human worth, dignity and privacy.
- To receive support, as carers and other support persons, to collaborate as far as is possible with support programs aimed at assisting the consumer to regain personal autonomy and empowerment.
- To be provided with information, education, training and support to facilitate the understanding, advocacy and care of those consumers they care for, and the contact details of carer community groups that can provide comprehensive information, education, support and assistance (e.g. [Helping Minds](#), [Carers WA](#), [MHM2](#), [MIFWA](#)).
- To be informed about any risks associated with clinical interventions and alternatives and given the opportunity to participate in the decision making about care.

With the consumers consent:

- To consult carers and other support persons about measures under consideration for the treatment or welfare of the consumer. Assist carers and other support persons in arranging support services such as respite care, counselling and community nursing facilities.
- To provide all carers and other support persons with general, non-personal, information about the consumer's mental health condition, treatment and ongoing care, even if consent is not given by the consumer.

Refer to the WA [Department of Health You Matter – a guideline to support engagement with consumers, carers, communities and clinicians in health](#)

Managers have the responsibility to:

- Include rights of carers and other support persons in their staff orientation program.
- Ensure sufficient information (e.g. posters, pamphlets, etc.) are available for staff to provide to consumers, carers and other support persons.

7. Compliance and Evaluation

NMHS MH contributes to an annual report to demonstrate compliance with the [Carers Recognition Act 2004](#) facilitated by the MHPHDS Safety, Quality and Performance Unit to NMHS.

Related internal policies, procedures and guidelines

[NMHS Carers Policy](#)

[NMHS MH Provision of Further Opinions Under the Mental Health Act 2014 Policy](#)

[NMHS Open Disclosure Policy](#)

[MH Goals of Patient Care Procedure](#)

References

[Carers Recognition Act 2004 \(including the Carers Charter\) \(WA\)](#)

[Carers Recognition Act 2010](#)

[Chief Psychiatrist's for Clinical Care Standard: Consumer and Carer Involvement in Individual Care](#)

[Department of Health You Matter – a guideline to support engagement with consumers, carers, communities and clinicians in health](#)

[Nominated Persons: How to make a Nomination and what it means](#)

[Mental Health Act 2014 \(Charter of Mental Health Care Principles\)](#)

[Department of Health Mental Health statements of rights and responsibilities](#)

[National Commission of Safety & Quality - Australian Charter of Healthcare Rights](#)

[WA Guardian & Administration Act 1990](#)

[National Standards for Mental Health Services](#)

[National Safety and Quality Health Service \(NSQHS\) Standards \(Version 2\)](#)

Useful resources (including related forms)

[NMHS MH Public Patient's Charter](#)

[Mental Health Commission \(WA\) Information for personal support persons brochure](#)

[The Royal Australian & New Zealand College of Psychiatrists](#)

Policy Sponsor	Adult Inpatient Medical Co-Director MHPHDS				
Policy Contact	Service Manager Adult Inpatient MHPHDS				
Date First Issued:	25/07/2014	Last Reviewed:	17/07/2023	Review Date:	17/07/2026
Approved by:	Mental Health Policy Coordination Committee			Date:	19/07/2023
Endorsed by:	Executive Director MHPHDS			Date:	17/07/2023
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☐  Std 5: Comprehensive Care ☐  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☒ Std 7: Carers ☐ Std 8: Governance, leadership and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Transfer of Care				
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Document Version Control			
Date	Version	Author	Notes
07/06/2018	1.2	A/Policy Officer	Amended review date to reflect review against MHA Act 23/11/2015. Repaired all hyperlinks
14/06/2019	2.0	Program Manager, Osborne Park Community Mental Health Service	Scheduled Review: Change of title – previously 'Patient And Carer Rights And Responsibilities', content revised to align with current legislation, updated references, hyperlinks and template, Change of policy sponsor - previously Deputy Executive Director.
13/09/2022	3.0	Manager Adult Inpatient Services MHPHDS	Scheduled Review: Scheduled Review, changed from policy to procedure, updated information as per overarching NMHS Policy, change of content order to enhance understanding of requirements in 1st and 2nd paragraphs of section - 5. Principles.
17/07/2023	3.1	Adult Inpatient Co-Directors MHPHDS	Scheduled review, update of carer and consent information in accordance with NMHS policy, hyperlinks updated.

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Appendix 1: Explanation of rights of carers, close family members or other personal support persons

The Mental Health Act 2014 provides that a mental health service must explain a patient's rights to them at various stages throughout the patient journey (including at admission, when an inpatient treatment order is made and when a community treatment order is made). Refer to [Mental Health Act 2014 s. 245](#).

The person responsible for giving the explanation must also ensure that a carer, close family member or other personal support person of the patient is provided with an explanation of the patient's rights.

It is immaterial that the patient might not have consented to the carer, close family member or other personal support person being provided with that explanation of rights.

Person's best interests

In deciding what is or is not in the best interests of a person, health services are to have regard to:

- the wishes of the person;
- the views of the person's enduring guardian or guardian, parent or guardian (if the person is a child), nominated person, carer and close family member; and any other matter that the health service considers relevant to making the decision. Refer to [Mental Health Act 2014 s. 7](#).

Rights of carers and close family members

Subject to any limitations regarding consent, any carer or close family member of a patient is entitled to be provided with information about the patient's treatment and care, including information about the following matters:

- the mental illness for which the patient is being provided with treatment or care;
- the grounds on which any involuntary treatment order was made in respect of the patient;
- the treatment and care proposed to be provided to the patient;
- any other options for the patient's treatment and care that are reasonably available;
- the treatment provided to the patient and the patient's response to that treatment;
- the seclusion of the patient;
- the use of bodily restraint on the patient; and
- the services available to meet the patient's needs.

Subject to any limitations regarding consent, any carer or close family member of a patient is entitled to be involved in:

- the consideration of the options that are reasonably available for the patient's treatment and care;

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- the provision of support to the patient; and
 - the preparation and review of any treatment, support and discharge plan for the patient.

Any carer or close family member of a patient is entitled to be provided with information about the patient's rights under the Mental Health Act 2014 and how those rights can be accessed and exercised.

Any carer or close family member of a patient is entitled to be provided with information about the carer's or close family member's rights under the Mental Health Act 2014 and how those rights can be accessed and exercised

Rights of the nominated persons

Unless the patient's psychiatrist reasonably believes it is not in the patient's best interests, a patient's nominated person is entitled to be provided with information about the patient's treatment and care, including information about the following matters:

- the mental illness for which the patient is being provided with treatment or care;
- the grounds on which any involuntary treatment order was made in respect of the patient;
- the treatment and care proposed to be provided to the patient;
- any other options for the patient's treatment and care that are reasonably available;
- the treatment provided to the patient and the patient's response to that treatment;
- the seclusion of the patient;
- the use of bodily restraint on the patient;
- the services available to meet the patient's needs.

Unless the patient's psychiatrist reasonably believes it is not in the patient's best interests, a patient's nominated person is entitled to be involved in:

- the consideration of the options that are reasonably available for the patient's treatment and care;
- the provision of support to the patient; and
- the preparation and review of any treatment, support and discharge plan for the patient.

Refer to [Mental Health Act 2014 s. 266](#).

A patient's nominated person is entitled to be provided with information about the patient's rights under the Mental Health Act 2014 and how those rights can be accessed and exercised.

Unless authorised to do so in another capacity, a patient's nominated person is not authorised to apply on behalf of the patient for admission or discharge by a mental health service or to make a treatment decision about the provision of treatment to the patient. The person responsible for providing the nominated person with information, or to involve the nominated person in the matter, shall continue to make reasonable efforts to do so until it is reasonable to

conclude that it is impossible for the nominated person to be provided with the information or involved in the matter.

Refer to [Mental Health Act 2014s. 268](#).

If a psychiatrist decides that a nominated person is not entitled to be provided with information or involved in a matter because it is not in the patient's best interests, he/she must advise the nominated person of the decision and the reasons for it. This advice must be filed in the patient record and a copy given to the patient. If the nominated person requests a written confirmation of the advice, the psychiatrist will provide the written confirmation, file a copy in the patient record and give a copy to the patient.

Refer to [Mental Health Act 2014s. 269, 270](#).

A psychiatrist may revoke a decision that a nominated person is not entitled to be provided with information or involved in a matter. If the nominated person previously requested to be provided with the information or involved in the matter, the psychiatrist must ensure that this is done as soon as practicable and a record of the provision of the information filed and a copy given to the patient.

Refer to [Mental Health Act 2014s. 271](#).

Appendix 2: Patient consent to disclosure to, or involvement by, carers or close family members

Whether a carer or close family member is entitled to be provided with information about the patient's treatment and care, or to be involved in matters relating to the patient's treatment and care, is dependent upon:

- the status of the patient under the [Mental Health Act 2014](#); and
- the capacity of the patient to consent. Refer to [Mental Health Act 2014](#) s. 286-289.

Type of patient	Capacity to consent	No capacity to consent
Voluntary patient	Carer / close family member has right to information or involvement, with patient consent	Carer / close family member has right to information or involvement, unless not in patient's best interests
Involuntary patient or mentally impaired accused	Carer / close family member has right to information or involvement, unless patient refuses consent and such refusal is reasonable	Carer / close family member has right to information or involvement, unless not in patient's best interests

The person responsible for providing any carer or close family member with information, or to involve any carer or close family member in a matter, shall continue to make reasonable efforts to do so until it is reasonable to conclude that it is impossible for any carer or close family member to be provided with the information or involved in the matter. Refer to [Mental Health Act 2014 s. 291](#).

If a psychiatrist decides that a carer or close family member is not entitled to be provided with information or involved in a matter because it is not in the patient's best interests, he/she must advise the carer or close family member of the decision and the reasons for it. This advice must be filed in the patient record and a copy given to the patient. If the carer or close family member requests a written confirmation of the advice, the psychiatrist will provide the written confirmation, file a copy in the patient record and give a copy to the patient. Refer to [Mental Health Act 2014 s. 292](#).

A psychiatrist may revoke a decision that a carer or close family member is not entitled to be provided with information or involved in a matter. If the carer or close family member previously requested to be provided with the information or involved in the matter, the psychiatrist must ensure that this is done as soon as practicable and a record of the provision of the information filed and a copy given to the patient. Refer to [Mental Health Act 2014 s. 294](#).